Abiraterone and prednisolone: metastatic hormone-sensitive prostate cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Abiraterone + Prednisolone

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.
- Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. electrocardiogram (ECG) if clinically indicated or with cardiac history.
- Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00577 v3 · pTreatment / supportive care · NCCP regimen and NCIS antiemetic guidance

Sources checked 2026-11-14 · SACTCheck v0.84.51 · Regimen-level drafts; prescriber and oncology-pharmacy verification required